6.6 Gastrectomy*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Rates abdominal pain 7/10 **Objective** • Upper abdominal incision • NG tube in place	**ACTUAL** Acute pain related to the surgical incision as evidenced by a pain score of 7/10.	• Pain at or below 3/10 within 24 hours.	• Give analgesia regularly as prescribed. • Do not reposition the NG tube; report if it moves. • Support the incision when she coughs. • Mobilize early.	• Considered met when pain is controlled.
Subjective • "I get dizzy and sweaty after eating." **Objective** • Symptoms 15–30 minutes after meals	**ACTUAL** Inadequate nutritional intake related to reduced gastric capacity and dumping syndrome as evidenced by post-prandial symptoms.	• Eats without dumping symptoms.	• Advise small, frequent meals rich in protein and low in simple sugars. • Suggest drinking between meals instead of with them. • Suggest lying down for 20–30 minutes after eating. • Explain the need for long-term vitamin B12 replacement, as prescribed.	• Goal reached when dumping symptoms are absent.
Subjective *No specific complaint* **Objective** • Major abdominal surgery • Drain in place	**RISK** Risk for infection as evidenced by risk factors: surgical wound and anastomosis.	• No infection or anastomotic leak.	• Watch temperature, wound and drain. • Report fever, tachycardia or increasing pain. • Use aseptic technique for wound care.	• Met if no infection develops.

*References for this care plan: [14], [32], [35], [37]. Full entries are listed in the ReferFull entries are listed in the References section of the CPMN book.